

So Someone You Love (Might Have) BPD?

How to raise it, what helps in the hardest moments, and how to look after yourself while you do it.



BEFORE YOU START

What this guide is, and what it is not

You cannot diagnose the person you love, and nothing in here will let you. What this guide can do is give you words for what is happening and something to try when it happens again.

What it is

- Orientation, drawn from published clinical research.
- Language for conversations that keep going wrong.
- Concrete things to say, and things to stop saying.
- Permission to look after yourself while you do it.

What it is not

- A diagnosis. Only a qualified clinician can make one.
- A treatment plan, or a substitute for one.
- A script that will work every time, because none exists.
- A reason to tell someone they have BPD.

If things are dangerous right now, skip ahead.

The crisis numbers are set out later in this guide, and they are worth finding before you need them rather than during. If someone is in immediate danger, call **988** in the US and Canada, or **911**.

SECTION ONE

Gently bringing up support

When someone you care about experiences intense, fast-shifting emotions or fears of abandonment, suggesting professional help can feel daunting. Bringing up options like Dialectical Behavior Therapy directly can sometimes trigger defensive reactions or fears of rejection if it is not approached thoughtfully. You might consider these strategies when opening a conversation.

- **Choose a calm moment.** You might wait to bring up therapy when both of you are feeling relaxed and grounded, rather than during or immediately after an emotional escalation.
- **Focus on shared goals, not labels.** Rather than framing therapy around a diagnosis or disorder, you could center the conversation on relief: helping them feel less overwhelmed, finding tools to ease emotional pain, or supporting harmony between you.

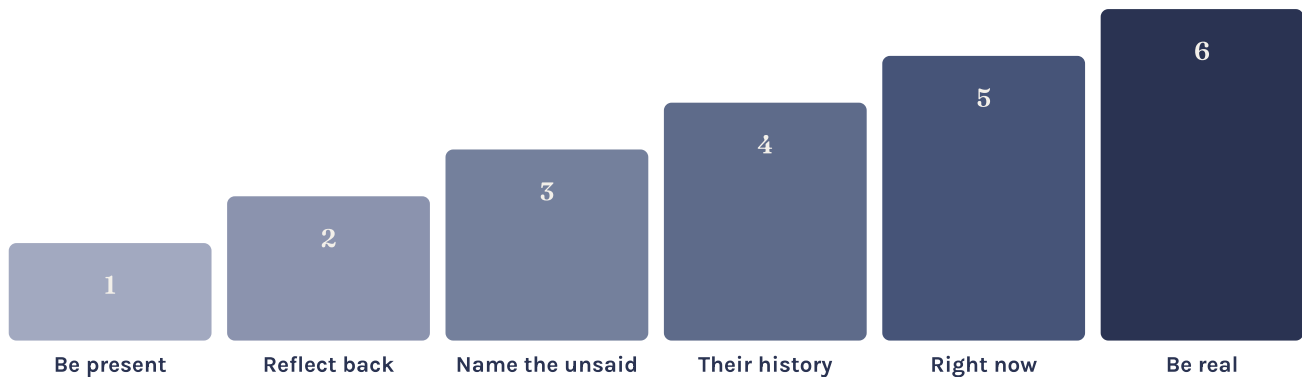
- **Use “I” statements and reassure security.** You might express your suggestions through your own perspective, for example “I love you and want us to have the best tools to support each other,” to lessen any fear that you are pulling away or judging them.
- **Suggest exploring options together.** You could offer to help look into resources or brief skills-focused programs together, allowing them to feel supported rather than singled out.

THE CORE SKILL

Six ways to validate

Validation is the single most useful thing in this guide, and it is the one most often misunderstood. It does not mean agreeing. It does not mean saying the behaviour was fine. It means showing that the feeling makes sense, which is what lets someone climb down far enough to talk. Marsha Linehan, who developed DBT, sets it out as six levels, each one going further than the last.

EACH ONE GOES FURTHER THAN THE LAST



You do not have to climb all six every time. Getting to level two reliably is worth more than reaching level six once.

1 Be present

Put the phone face down. Stay in the room. Do not multitask this.

“I’m here. I’m listening.”

2 Reflect back what you actually heard

Say it back without adding your version of events, and without the word “but” on the end.

“So when I checked my phone, it felt like I’d stopped listening.”

3

Name what they have not said

Guess at the feeling underneath and let them correct you. Being corrected is useful; it means they are engaging rather than escalating.

"I'd guess that felt like you weren't worth my attention. Am I close?"

4

Make sense of it from their history

Their reaction is often a reasonable response to something that happened before, arriving in the wrong decade.

"Given how many people have walked out on you, of course you're watching for it."

5

Make sense of it from right now

Sometimes no history is needed, because anybody would feel this. Say so.

"You waited three hours with no message. Anyone would be climbing the walls."

6

Be real with them

Treat them as an equal rather than as something fragile. Handling someone with tongs is its own kind of insult, and they can always tell.

"I'm frustrated too. I still want to sort this out with you."

IN PRACTICE**Instead of this, try this**

Six situations that come up again and again. The left-hand line in each is not a straw man; it is the reasonable, well-meant thing most people say, and it is the thing that reliably makes the next ten minutes worse.

They accuse you of not caring, or of being about to leave**INSTEAD OF**

"That's not true. After everything I've done for you?"

TRY

"I'm not going anywhere. I can hear how frightened you are that I might be."

Defending your record is a debate about facts. The fear is not about facts, so the debate cannot be won.

They are spiralling over something that looks small to you

INSTEAD OF

“You're overreacting. This isn't a big deal.”

TRY

“This is landing really hard. What's the worst part of it?”

Size is the wrong argument. Asking which part hurts most moves them from the feeling into describing it, which is itself calming.

They say something cruel in the middle of an argument

INSTEAD OF

“Don't you dare speak to me like that.” Or saying nothing at all and swallowing it.

TRY

“I want to keep talking about this, and I can't while I'm being spoken to this way. I'm going to take fifteen minutes and then I'm coming back.”

Both halves matter. The limit, and the promise that you are returning. Leaving without the second half confirms the fear.

They talk about hurting themselves

INSTEAD OF

“You're only saying that to get a reaction.”

TRY

“I'm taking that seriously. Are you safe right now? I'd like to call 988 and sit with you while we do it.”

Recurrent suicidal behaviour is one of the nine diagnostic criteria, not a debating tactic. Treat every statement as real, every time, and hand it to people trained for it. This is not yours to carry alone.

You want to raise the idea of therapy

INSTEAD OF

“I think you need help. I've been reading, and I think you have BPD.”

TRY

“I've read about some skills for the kind of overwhelm you describe. Would you want to look at them with me?”

Lead with relief, not with a label. A diagnosis delivered by a relative lands as an accusation, however kindly it is meant.

The next morning, after a bad night

INSTEAD OF

Pretending it did not happen, and hoping it does not happen again.

TRY

“Last night was hard for both of us. I love you. Can we work out what would help next time?”

Repair is the part most families skip. Doing it while everyone is calm, and without relitigating who was right, is what stops the same night repeating.

SECTION TWO

What to do in moments of distress

Supporting a loved one through intense emotional peaks requires practical self-awareness and steady responses. These are tangible steps you can take to help manage high-stress moments safely.

- **Co-regulate and use sensory grounding.** When stress is very high, logical reasoning often takes a backseat to intense emotional pain. Instead of trying to debate or resolve the issue immediately, you might offer gentle sensory tools, such as bringing them a glass of ice water, offering a cool washcloth, or stepping outside for fresh air, to help soothe their physical nervous system without pressure.
- **Validate emotions while setting boundaries.** You can validate their emotional experience, for example “I can see how hurt and overwhelmed you feel right now,” without agreeing with hurtful behaviors or compromising your own well-being. Setting gentle, clear boundaries, such as taking a temporary fifteen-minute break to cool down, protects both of you and supports long-term emotional stability.

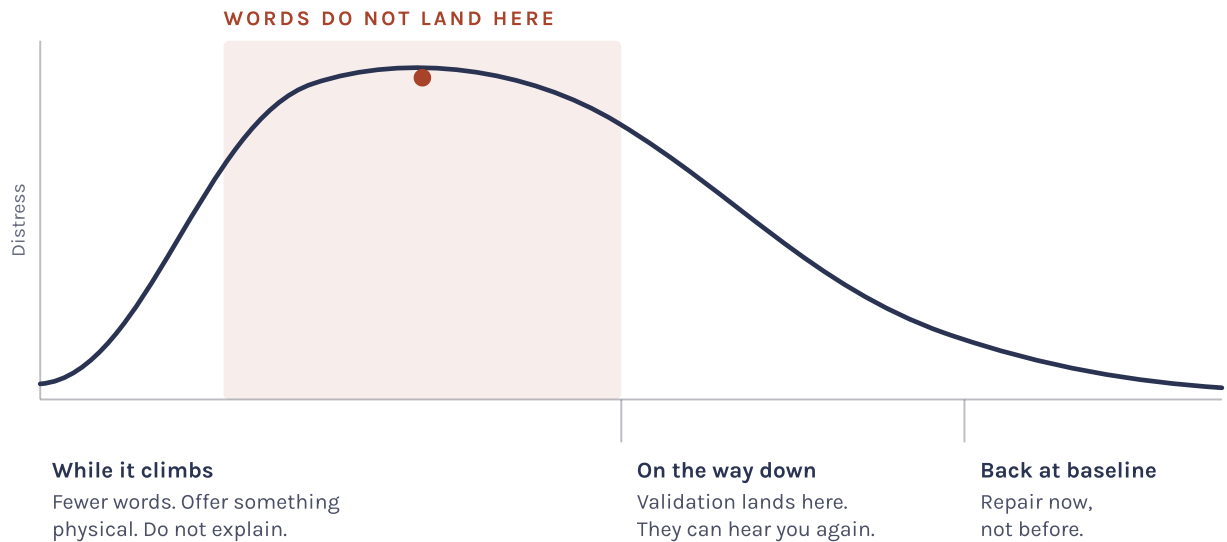
You are not responsible for regulating someone else's emotions.

You can be steady, and you can be kind, and you can still be unable to fix it. That is not a failure on your part. It is the reason structured treatment exists.

WHEN IT IS HAPPENING

What to do, in order

In a high-distress moment your instinct will be to explain, and explaining is the one thing that will not work. The reason is in the shape of the thing.



Schematic, not measured data. The shape is the point: distress climbs fast and comes down slowly, and the window where talking works opens on the way down, not at the top.

So the order matters more than the words. Work down this list.

Is anyone in immediate physical danger?

If yes, stop here

Call **988**, or **911** if there is immediate risk to life. Say that the emergency involves a mental health crisis. Nothing further down this page comes first.

Can they take in words right now?

IF NO

Use fewer words, not better ones. Offer something physical: a glass of ice water, a cold cloth, stepping outside. Do not explain, correct, or defend yourself. You are waiting for their body to come down, not their argument.

Have you validated before offering any solution?

DO THIS FIRST

Levels one to three from the previous page. Problem-solving offered before validation reads as dismissal, and you will have to start again.

Are you being spoken to in a way you cannot accept?

IF YES

Name the limit, give a specific length of time, leave, and then actually come back when you said you would.

It has passed. What now?

LATER, NOT NOW

Repair while you are both calm. Do not reopen who was right. Ask what would help next time, and write the answer down somewhere you will both find it.

If they are in immediate danger

Recurrent suicidal behavior and self-harm are part of the diagnostic picture for BPD. Keep these where you can find them quickly.

- **988 Suicide & Crisis Lifeline.** Call or text **988**. Free and confidential, 24/7 in the US and Canada. You can call about someone else.
- **Crisis Text Line.** Text **HOME** to **741741** to reach a crisis counselor, 24/7.
- **The Trevor Project** (LGBTQ youth). Call **866-488-7386** or text **START** to **678-678**, 24/7.
- If there is immediate risk to life, call **911** and say that the emergency involves a mental health crisis.

THE PART EVERYONE FINDS HARDEST

Boundaries that hold

A boundary is a statement about what you will do. It is not a rule you are imposing on somebody else.

This distinction sounds like word games and it is not. A rule invites a negotiation you cannot win, because you cannot enforce it. A boundary needs no agreement from anyone, because the only person it commits is you.

Being shouted at

A RULE, WHICH WILL BE TESTED

"You need to stop shouting at me."

A BOUNDARY, WHICH YOU CAN KEEP

"If the shouting carries on I'm going to sit in the garden for fifteen minutes, and then I'll come back in."

Calls and messages through the working day

A RULE, WHICH WILL BE TESTED

"You can't keep calling me at work."

A BOUNDARY, WHICH YOU CAN KEEP

"My phone is off between nine and five. I'll call you at lunchtime, every day."

Being the only support they have

A RULE, WHICH WILL BE TESTED

"You have to go to therapy."

A BOUNDARY, WHICH YOU CAN KEEP

"I'll come with you to the first appointment. I'm not able to be the only person you lean on, and I don't want to be the reason you don't get better help."

A boundary you do not keep is worse than no boundary at all.

Announcing a limit and then not holding it teaches that limits are a stage in the argument. Only set ones you are genuinely willing to follow through on, every time, including at eleven at night when you are exhausted.



The people supporting someone with BPD need supporting too. That is not self-indulgence, it is what makes the support sustainable.

SECTION THREE

Long-term strategies for you

Caring for someone with intense emotional needs works best when you also prioritize your own emotional health and clear communication.

- **Build your own support system.** You might seek out your own therapist, support group, or trusted friends, so that you have a dedicated space to process your own feelings rather than carrying them alone.
- **Practice steady patience.** Progress and skill-building often take time and involve setbacks. Acknowledging small positive changes helps reinforce growth for both of you.
- **Explore flexible resource options.** If full-scale therapy feels like too big a commitment for them right now, you might gently introduce shorter skills-based workbooks or brief community groups as a low-pressure starting point.

Families can learn the same skills.

Family Connections, a program that adapts DBT for relatives, has been trialed with 121 family members, with promising but not yet conclusive results on caregiver burden. The skills in guide two are not only for the person with the diagnosis.

YOUR OWN HEALTH

An honest check

WHAT IS ACTUALLY YOURS TO CARRY

Theirs	Yours	Their clinician's
Whether they accept help Their own recovery What they say in a crisis Doing the work of therapy	Staying steady Saying what you will do Keeping the limits you set Looking after yourself Calling 988 if it is needed The only column you can act on	Making the diagnosis Choosing the treatment Assessing risk Adjusting what is not working

Most of the exhaustion in supporting someone comes from working the first and third columns, where nothing you do can reach.

Caregiver strain is measurable and it creeps up slowly. Read these and count the ones that are true of the last month, not of your life in general.

- ☐ I have cancelled my own plans more than twice because of a crisis at home.
- ☐ I pick up my phone with a feeling of dread.
- ☐ I have stopped telling my friends what is actually going on.
- ☐ I am sleeping badly, or sleeping with the phone on loud beside me.
- ☐ I cannot remember the last thing I did purely because I wanted to.
- ☐ I manage my own face and tone constantly, at home, to avoid setting something off.
- ☐ I believe that if I stopped doing this, they would not survive it.

Three or more is worth taking to someone.

The last one especially. Believing you are the thing keeping another person alive is the point at which you need support of your own, not more resolve. That belief is also usually not true, and carrying it will eventually make you less use to the person you are trying to help, not more.

Meet Marley



This project started with one family trying to make sense of a diagnosis.

Marley grew up inside a family where emotions ran high and nobody had a word for why. That question led her to spend a summer buried in clinical research on borderline personality disorder, and to build this series of guides for other families doing the same thing hers did.

Each guide is written from the peer-reviewed literature rather than from anecdote, and every claim is traceable to the sources listed at the back. They are free, and they are meant to be shared with the people who need them.

“Nobody in my family had the language for what was happening. These guides are the thing I wish someone had handed us.”

SOURCES

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The Family Connections figure is cited from a trial of 121 family members and is described in the source as promising rather than conclusive. Crisis numbers were verified as current at the time of writing.